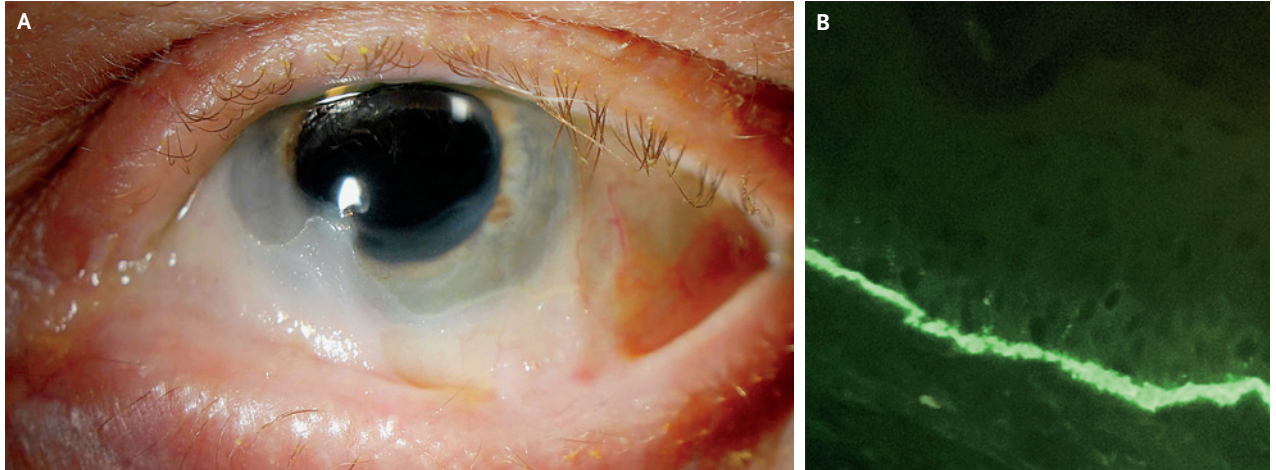


## IMAGES IN CLINICAL MEDICINE

Stephanie V. Sherman, M.D., *Editor*

## Symblepharon in Linear IgA Bullous Dermatositis



A 73-YEAR-OLD MAN WITH HYPERLIPIDEMIA AND CHRONIC KIDNEY DISEASE presented to the ophthalmology clinic with a 5-month history of blurry vision, photophobia, and burning in both eyes. He also reported the development of lesions on his neck and scalp over the same period. He reported that he had taken no new medications and that he had not recently used antibiotics. On physical examination, scattered vesicles and bullae were found on his neck, scalp, arms, and back. Visual acuity was 20/30 in his right eye and 20/70 in his left eye. Filmlike adhesions were observed between the bulbar and palpebral conjunctivae in both eyes, findings consistent with symblepharon (Panel A). A biopsy specimen of a lesion on one of his arms was obtained in the dermatology clinic; direct immunofluorescence revealed a linear band indicating the presence of IgA antibodies to antigens in the basement membrane (Panel B). A diagnosis of idiopathic linear IgA bullous dermatosis with ocular involvement was made. Treatment was initiated with dapsone and with systemic and topical glucocorticoids, topical cyclosporine, and eye lubricants. The patient was lost to follow-up. Patients with autoimmune blistering disease should be evaluated for ocular involvement.

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